

On-scene Authority for Patient Care

Standard:

Establish the clinical hierarchy of authority for on-scene patient care.

Purpose:

Credentialed Providers within the ATCEMS System are responsible for providing patient care in accordance with the prescribed protocols, standards and procedures. However there may be times when providers disagree about the care being delivered. Patient safety is the responsibility of every provider and any concerns should be immediately brought to the attention of other caregivers at the scene. In ANY disagreement regarding circumstances relating to patient care a professional demeanor and focus on the best interest of the patient is paramount. In order to maintain an orderly scene and allow rapid resolution of conflict a hierarchy of clinical responsibility must be established.

Application:

- 1) In the event of conflicting approaches to providing patient care, extraction, or transport, it is the responsibility of the on-scene credentialed providers to reach consensus as to the most appropriate care for the patient(s). In the event of unresolved conflict, the senior credentialed provider on-scene has final authority and responsibility for decisions regarding patient care. If there is a conflict involving a supervised provider (Cadet/Student/Candidate) the assigned training officer has authority (at their level of credential) and should be consulted.
- 2) Seniority of Credentials (in descending order) is:
 - a) EMS System Medical Director or designee
 - b) On-Line Medical Consultation Physician
 - c) On-scene Physician (In accordance with Physician on Scene Clinical Standard)
 - d) Paramedic Practitioner Credentialed PL-7
 - e) Credentialed PL 6
 - f) DMO or Training Captain PL 5 on Transporting Unit
 - g) Credentialed PL 5
 - h) Credentialed PL 4
 - i) Credentialed PL 3
 - j) Credentialed PL 2
 - k) Credentialed PL 1
- 3) All significant or unresolved conflicts regarding on-scene management of patients should be reported via the appropriate clinical event review process (Medical Officer) and will be retrospectively reviewed in accordance to each organization's review process.
- 4) If any provider, regardless of credential, feels the conflict negatively impacted patient care, the incident should be reported to the Office of the Chief Medical Officer as soon as practical without causing an additional impediment to care.